

MEDICAL REPORT

- **Patient Name:** John Doe
 - **Date of Birth:** 07/15/1970
 - **Medical Record Number (MRN):** 123456789
 - **Gender:** Male
 - **Age:** 55
 - **Date of Report:** August 26, 2025
-

SUMMARY

Mr. John Doe, a 55-year-old male, presents for his scheduled six-month follow-up visit. He was initially diagnosed with Type 2 Diabetes Mellitus (T2DM) in 2010. His most recent lab results indicate elevated HbA1c and fasting blood glucose levels, suggesting poor glycemic control. He reports experiencing recent symptoms of fatigue, increased thirst, and occasional blurry vision. He also has a history of hypertension and dyslipidemia, which are being managed.

CLINICAL ENCOUNTER

Date of Visit: August 25, 2025

Chief Complaint: "Follow-up for diabetes management. Feeling more tired and thirsty lately."

History of Present Illness (HPI): The patient reports that his blood sugar readings at home have been consistently high in the mornings, typically in the 180-220 mg/dL range. He admits to some dietary indiscretion, particularly with sweetened beverages. He reports increased fatigue and is getting up to urinate more frequently at night (3-4 times per night). He also mentions occasional blurry vision, which he attributes to eye strain from computer use. He denies any chest pain, shortness of breath, or new numbness or tingling in his hands or feet.

Past Medical History:

- **Type 2 Diabetes Mellitus (T2DM):** Diagnosed in 2010.
- **Hypertension:** Diagnosed in 2012, stable on current medication.
- **Dyslipidemia:** Diagnosed in 2015, stable on current medication.
- **Allergies:** No known drug allergies.

Medications:

- Metformin 1000 mg, twice daily
- Lisinopril 20 mg, once daily
- Atorvastatin 40 mg, once daily
- Aspirin 81 mg, once daily

Physical Examination:

- **General:** Well-developed, well-nourished male in no acute distress.
- **Vitals:** BP 138/86 mmHg, HR 78 bpm, RR 16 bpm, Temp 98.6°F. BMI: 31.2 kg/m².
- **Cardiovascular:** Regular rate and rhythm. No murmurs, gallops, or rubs.
- **Pulmonary:** Lungs clear to auscultation bilaterally. No rales, rhonchi, or wheezes.
- **Neurological:** Alert and oriented. Cranial nerves II-XII intact. Sensation to light touch and pinprick is intact in all four extremities. Patellar and Achilles reflexes are 2+ bilaterally.
- **Feet:** Skin is intact. No ulcers, calluses, or deformities noted. Monofilament test reveals some decreased sensation in the toes bilaterally.

Review of Systems:

- **General:** Reports fatigue.
- **Endocrine:** Reports increased thirst and frequent urination.
- **Ophthalmic:** Reports blurry vision.
- **Gastrointestinal:** Denies nausea, vomiting, or diarrhea.
- **Genitourinary:** Reports frequent urination, particularly at night.
- **Musculoskeletal:** Denies joint pain or swelling.

LABORATORY & DIAGNOSTIC RESULTS

Date of Labs: August 24, 2025

Chemistry Panel:

- **Fasting Glucose:** 198 mg/dL (High - Reference: 70-99 mg/dL)
- **Creatinine:** 1.1 mg/dL (Reference: 0.7-1.3 mg/dL)
- **eGFR:** 78 mL/min/1.73m² (Reference: >60 mL/min/1.73m²)

Lipid Panel:

- **Total Cholesterol:** 215 mg/dL (High - Reference: <200 mg/dL)
- **HDL Cholesterol:** 42 mg/dL (Low - Reference: >45 mg/dL)
- **LDL Cholesterol:** 140 mg/dL (High - Reference: <100 mg/dL)
- **Triglycerides:** 188 mg/dL (High - Reference: <150 mg/dL)

Hemoglobin A1c (HbA1c):

- **HbA1c:** 8.8% (High - Reference: <5.7%)

Urine Microalbumin/Creatinine Ratio:

- **Ratio:** 45 mg/g (High - Reference: <30 mg/g)
-

ASSESSMENT & PLAN

Diagnosis:

- **E11.8:** Type 2 diabetes mellitus with unspecified complications.
 - *Rationale:* The patient's consistently high blood glucose levels (HbA1c of 8.8%) indicate uncontrolled T2DM. His symptoms of polyuria, polydipsia, and blurry vision, as well as the lab findings of elevated urine microalbumin and decreased sensation on monofilament testing, suggest the presence of diabetes-related complications, though they are not fully specified with a more detailed ICD-10 code.
- **I10:** Essential (primary) hypertension.
- **E78.5:** Hyperlipidemia, unspecified.

Plan:

1. **Diabetes Management:**
 - **Medication:** Increase Metformin to 1000 mg three times daily. Discussed the need for potential insulin initiation at the next visit if glycemic targets are not met.
 - **Diet:** Patient counseling on the importance of reducing carbohydrate intake, especially from sugary drinks. Referral to a registered dietitian for medical nutrition therapy.
 - **Monitoring:** Advised patient to check blood glucose levels more frequently (4 times per day) and to keep a detailed log.
2. **Cardiovascular Risk Reduction:**
 - **Lipid Management:** Discussed the importance of diet and exercise. Will continue Atorvastatin and will re-evaluate lipid panel in 6 months.
 - **Blood Pressure:** BP is borderline. Encouraged lifestyle modifications. Will continue Lisinopril and monitor closely.
3. **Complication Screening & Follow-up:**
 - **Nephropathy:** Elevated urine microalbumin is a concern. Advised on a low-sodium diet. Will repeat urine microalbumin/creatinine ratio in 3 months.
 - **Neuropathy:** Monofilament test shows signs of peripheral neuropathy. Advised on daily foot checks and to wear comfortable, well-fitting shoes. Referred to podiatry for a comprehensive foot exam.
 - **Retinopathy:** Referred to an ophthalmologist for a comprehensive dilated eye exam due to reported blurry vision and T2DM diagnosis.
4. **Patient Education:**

- Provided educational materials on T2DM management, including diet, exercise, and symptom recognition.
 - Scheduled a follow-up appointment in three months to review blood glucose logs, lab results, and adjust the treatment plan as needed.
-